

Statement of Work

INTRODUCTION

The National Institutes of Health (NIH) Clinical Center (CC) is America's clinical research hospital and the world's largest hospital dedicated totally to clinical research. Through clinical research, promising discoveries in the laboratory are translated into better health and health care for all. Currently, there are approximately 1,500 active clinical research studies.

The vision of the NIH Clinical Center is to serve as the nation's premier research hospital for conducting clinical research to improve the health of humankind. It also serves as a national resource for clinical research by developing diagnostic and therapeutic interventions; enhancing systems to ensure the safe, efficient, and ethical conduct of clinical research; training clinical researchers; and leading the response to the nation's public health needs.

In 2005, the Mark O. Hatfield Clinical Research Center was opened. This state-of-the-art facility is home to inpatient units, day hospitals and research labs and connects to the existing Warren Grant Magnuson Clinical Center, which opened its doors to patients in 1953. Together, the Magnuson and Hatfield centers form the NIH Clinical Center, the world's largest clinical research complex, servicing a dual role: providing humane and healing patient care and an environment for clinical researchers to advance clinical science.

BACKGROUND

The Clinical Center at the National Institutes of Health (NIH) in Bethesda, Maryland, is the nation's largest hospital devoted to clinical research with patients from all over the United States and some from abroad. The Clinical Center provides medical care and research services to support patients participating in over 1500 active protocol sponsored by the NIH Institutes. The NIH Clinical Center has a requirement to provide consistent and reliable Medical Support Assistant staffing for the Nursing Department and the Patient Support Services Department (PSSD).

PSSD reports to the Clinical Center Office of the Director. PSSD is responsible for front-line customer service, reception, admission registration, creating patient vouchers, processing of patient travel invoices, and processing of appointments for various treatment and service areas.

SCOPE OF WORK

TASK 1: Nursing Department MSA

The NIH Clinical Center has a need for dedicated and committed individuals to perform in a health care environment that is aligned to the NIH mission. The Contractor shall, on a consistent basis, provide contract personnel to provide Medical Support Assistant staffing services. This may include a contractor staying beyond their assigned tour of duty to meet the program of care needs. The Contractor shall be

responsible for ensuring that staff assigned to this contract will provide consistent and reliable services meeting all the requirements of the specific position and meeting program needs.

The Contractor shall implement and administer a Structured, Stackable Shift Differential Pay Plan (the “Plan”) for contract personnel assigned under this SOW. The Plan shall be consistent with applicable Wage Determinations and the terms of this contract and shall be used to recruit and retain qualified personnel for off-shift coverage (evenings, nights, weekends, and holidays). The Plan must include the elements below and be included in the Contractor’s staffing, timekeeping, and invoicing systems.

Differential Categories and Eligibility — Differential pay shall be applied only to time actually worked in the categories defined below. Differential categories are additive (stackable) up to the caps specified in this clause. Ensure differential-eligible hours are properly authorized and recorded; review and sign weekly differential hour summaries for assigned staff prior to submission to Contractor payroll.

- a) The Government’s approved baseline differential schedule is:

Evenings (1500-2300)	\$3.50/hr
Nights (2300-0700)	\$3.50/hr
Weekends (Sat 7am-Mon 7am)	\$5.00/hr
Federal Holidays:	1.5x Base hourly rate
Stacked Differentials Cap	\$12.00/hr

- b) Stacking Rules and Caps — Differentials for separate eligible categories may stack (e.g., Night + Weekend). Stacked differentials shall not exceed \$12.00 per hour in total without prior written approval by the Contracting Officer. Holiday premiums shall stack with other differentials subject to the cap above.
- c) Application and Documentation — Contractor shall apply differential only for hours actually worked and documented in the Contractor’s timekeeping system. Timecards and electronic timesheets shall flag differential-eligible hours and record which differentials were applied. The Contractor shall maintain backup documentation (approved schedule, clock-in/clock-out records, and supervisor sign-off) for all differential payments for a minimum of three (3) years and shall make them available to the Government upon request.
- d) Billing and Invoicing — Differential pay amounts must be separately itemized on each monthly invoice (see Section 16). Each invoice shall include: (1) total differential hours by differential category (evening, night, weekend, holiday), (2) differential dollars by category, (3) any stacked differential dollars, and (4) supporting summary timesheets (redacted if necessary) for sample verification.
- e) Changes & Wage Determination Compliance — The Plan must comply with the currently-incorporated Wage Determination (including revisions). If a Wage Determination revision or other

regulatory change alters pay requirements, the Contractor shall promptly notify the Government and propose any needed Plan adjustments for approval.

- f) **Dispute/Correction** — If the Government identifies invoices or pay application errors, the Contractor shall correct billing errors within 30 calendar days and provide a remediation plan if systemic errors are identified.

Personnel working in all the areas will be interacting in-person and by phone with patients, visitors, physicians, nurses and other staff and it is important that the contractor personnel provide outstanding, consistent customer service for all customers. Patients that are seen at the NIH Clinical Center may be anxious, fearful, sick, have physical deformities, etc. In addition, many of the patients and families are not from the local area and this sometimes creates additional anxiety. It is extremely important that the contract personnel maintain a consistent high level of customer service and convey understanding, responsiveness, care, and compassion.

The duties listed below are a general list of the requirements for this contract. The Medical Support Assistants will only be performing the duties for the specific area assigned. Not all duties below apply to all areas. Contract staff will be expected to learn to work in multiple locations and will be required to float to meet the program staffing needs for that day.

Contract personnel shall have a minimum of 1 year experience in a health care setting working directly in-person with patients and have basic knowledge of medical terminology. The government is not responsible for training any contractor who has never worked in a health care setting.

The government will provide all training required to perform the duties of the specific Medical Support Assistant role at the NIH Clinical Center.

All duties shall be performed accurately and timely and all interactions with customers (internal and external) to the Clinical Center must be performed in a professional manner demonstrating outstanding customer service.

The functions performed by the Medical Support Assistants are critical to supporting the clinical research mission of the NIH Clinical Center. The personnel assigned to perform these functions will be interacting directly with patients, visitors and staff. They may be the first and lasting impression that our patients and visitors have of the NIH Clinical Center, and it is important that our customers always leave a service area with a positive experience.

- Scheduling Activities:** Schedules patient appointments in the computerized appointment scheduling system. Appointments must be entered accurately and according to the guidelines for the specific area assigned. All visits for the day must be statused (the patient status is marked as arrived, no-showed or canceled) in the scheduling system before the end of each day. Complete requests for new or modifications to existing schedule templates.

- Admissions and Outpatient Registration Activities:** Transcribe requests for patient admission; pre-register patients for admission; admit inpatients and register new outpatients. All these activities are entered in the NIH Clinical Research Information System (CRIS). Attention to detail is required in data

entry process to ensure that no duplicate records are created for patients and the demographic information is entered timely and accurately. In addition, patients are to be seen in a timely manner and personnel shall have an understanding of the consent process and be able to answer questions that new patients may have about consents and general information regarding the Clinical Center. Apply an accurate patient identification wristband to all patients. Assist physicians, nurses and patients with forms and other procedures related to patient expirations.

- Reception Activities: Prepare areas for the workday; receive and direct telephone calls; provide reception; make arrangements for interpreter services; close area for the work day according to the standard closing procedures for that area. Apply an accurate patient identification wristband to all patients. Accurate and complete documentation of calls or requests; prioritization of requests and timely response.

- Patient Record Activities: Prepare any medical record or visit paperwork for the patient visit; assemble inpatient chart; disassemble inpatient chart for filing in medical record; submit any patient records to the Health Information Management Department after correct patient label applied.

- Clinical Support Activities: Perform monthly safety inspections; and provide requested patient identification materials (labels, wristbands, etc.).

- Equipment, Material and Supply Support Activities: Request and stock appropriate supplies and forms; request and stock office supplies and forms; acquire meals and snacks for patients; monitor patient care nutrition and specimen refrigerator temperatures on a daily basis.

- Travel and Local Transportation Services Support Services: Transcribe requests for patient travel authorizations into the NIH Business Systems (NBS); issue travel authorizations and vouchers to the government approving official for approval before disbursement to patient or authorized guardian; scan all travel documents into the appropriate electronic patient record; reconcile lodging and taxi invoices against travel authorizations; arrange authorized patient travel and transportation.

- Statistics, Reporting and Other Administrative Activities: Enter data, produce, and/or distribute various reports; collect and enter patient wait and processing time statistics; filing, document topics discussed in meetings and orientation; issue keys and control key inventory. Requires knowledge of Microsoft computer applications, scanning, copying, and filing.

Actions in the Event of Non-Compliance with the Duties Above: The government reserves the right to request the removal and replacement of any of the Contractor's employees if the employee fails to successfully complete orientation, fails to complete his/her duties accurately and timely or for any other reasonable cause. If removal occurs within the first two (2) weeks of placement, the Government will not be held responsible for payment of contractor services. If the contractor staff fails to deliver or perform the services within the specified orientation period or any extension, the Contractor shall, in place of actual damage pay to the Government as fixed, agreed upon what costs are associated with the training of the contractor staff by the calendar year the training took place or concurrence period decided upon by the COR.

The NIH COR or designee will coordinate as necessary with the NIH Contracting Officer (CO) and make such a request only to the Contractor's Project Manager and/or designated representative.

Language Requirements: The Contract Personnel shall communicate effectively through oral and written communication. In addition, Contract Personnel shall have the ability to speak, read, write and understand the English language.

Physical Requirements: Duties for most areas require walking, carrying medical records, medical equipment, etc. up to about 50 pounds. A significant part of the job involves working with computers.

ONSITE SUPERVISOR RESPONSIBILITIES: The Contractor on-site supervisor shall be responsible for ensuring that all positions are filled by a trained contractor at the required times. That means the Contractor must have coverage for planned and unplanned leave and to ensure continuity of services.

On-Site Hours: 7:30 am – 4:00 pm Monday – Friday

It is recommended that the supervisor alter schedule, as needed, to assess performance of contract staff working on the off-shift

RECRUITMENT

- Provide the government Contracting Officer Representative (COR) and the Government Supervisor of the location resumes of qualified applicants within 5 business days of request.
- All qualified applicants must be pre-screened by the contractor and the on-site contract supervisor prior to providing resumes to the government. Applicants need to understand the salary, benefits, tour of duty, and emergency personnel status, prior to providing the government resumes.
- On-site supervisor will coordinate with government supervisors/CORs telephone/virtual screening appointments for qualified applicants.
- On-site supervisor will be present and participate in telephone/virtual screening calls with government supervisors.
- COR will notify the on-site supervisor of contractor selections and on-site supervisor will initiate their company's process for offering the contractor the position.
- Notify the COR if applicant accepts or declines the contractor's position.

NEW HIRE CONTRACTORS

- On-site supervisor must ensure that contractor completes the required paperwork for the Division of Personnel Security and Access Control (DPSAC) to satisfy all of the federally mandated requirements for obtaining a Personal Identity Verification (PIV) ID badge and/or background investigation required for a position with the U.S. Government.
- Ensure that the contractor completes the new hire documents which include: mandatory medical clearance documentation, the non-disclosure form, physical indicating no restrictions to work, drug testing and IT security training requirements.
- Provide the COR with the completed new hire documents within 3 weeks of offering the applicant the position.

- Meet with all new contractors on the first day they start their position and review time and attendance and emergency personnel designation procedures.
- Coordinate with the Patient Support Services Department training coordinator new employee orientation.
- Meet with the CCND nurse educator to review how the new contractor is doing during initial orientation.
- Meet with government supervisor where contractor is assigned to work a minimum of once a week to discuss how new contractor is doing with role based orientation.

STAFFING

- Understand how staffing is arranged on a weekly, monthly, quarterly basis for each location with a mixed staffing model of both government and contract positions.
- Understand that the contract supervisor cannot independently make staffing decisions without consultation with the government supervisor so as not to negatively impact the patient service operations.
- Notify government supervisor and COR of any unplanned leave (2 hours before the start of the shift) and tardiness (as soon as known) and plan for coverage. This includes weekends, holidays and evening/night shifts.
- Coordinate with government supervisor changes to daily staffing when there is no coverage.
- All scheduled leave, including leave around the Holidays, must be coordinated and approved by the government supervisor of the location to ensure there is appropriate staffing coverage.
- Notify COR if contractors have leave that they must take by a certain period of time, so it can be planned for, in advance.
- Implement a central sign-in log or electronic system for ensuring that all contract staff are at their duty stations ready to start work at the beginning of their shifts.
- Ensure that contract staff stay at their duty stations until the end of their shift.

If they need to leave for an emergency, coverage must be provided prior to their departure.

- The on-site supervisor must provide fully trained staff coverage for both unplanned and scheduled leave.

TRAINING

- Attend Clinical Center (CC) New Employee Orientation and CCND New Employee Orientation.
- Complete the CC New Employee Mandatory computer-based training modules and yearly CC Mandatory computer-based training modules.
- Learn and understand the functions that the contract staff will have to perform in each location and the deliverables and benchmarks required.
- Learn and understand The Joint Commission standards that apply to the contractors and ensure compliance with these standards.
 - o CRIS (electronic medical record)
 - o Enterprise Scheduling System
 - o Institute specific scheduling systems
 - o Admissions/Travel/Voucher request system

- o i-Med consent system
 - o Visual Supply Catalogue
 - o Credentialing Database
 - o Other applications, as required
- Ensure all contract staff complete CC Mandatory Clinical Center new employee orientation, CCND new employee orientation, new employee mandatory computer based training and yearly Clinical Center mandatory training.
 - Ensures all mandatory training requirements, throughout the year, are completed and turning in the certificates for all contractors to the COR by the deadline required by the COR.
 - Provide refresher training to identified contractors when the performance requirements are not being met.

MEETINGS

- Attend meetings with government supervisors and other Institute or department stakeholders as requested by the government supervisor or COR, if it relates to the responsibilities of the contract MSA.
- Meet weekly with the CORs for this contract to discuss recruitment, issues related to performance or staffing, and any new process changes that may impact the contractors

PERFORMANCE MANAGEMENT

- Ensure that all contract staff perform the duties of their position and meet required timelines for completion
- Provide refresher training, as needed. If training must be provided by the government, inform the COR of need for refresher training
- Obtain feedback from government supervisors and COR for contractor evaluations
- Give each contractor a mid-year and end of year evaluation and provide COR a copy of the end of year evaluation
- Review and approve all contractor timecards. Ensure differential-eligible hours are properly authorized and recorded; review and sign weekly differential hour summaries for assigned staff prior to submission to Contractor payroll.

REPORTS

Provide COR all required reports as listed in the SOW by the due date

MANDATORY VACCINATIONS

Ensure that all contract staff comply with any hospital mandatory vaccination

TASK 2: PSSD MSA

Independently and not as an agent of the Government, the Contractor(s) shall provide consistent contract personnel to deliver administrative support to admissions, inpatient/outpatient care areas, and other patient support service areas within the Clinical Center. These contract employees may also be required to float to another unit within the PSSD areas of responsibility.

PSSD Medical Support Assistant (MSA) must perform the following duties:

- Customer Service: Position requires direct contact in-person and by phone with patients, physicians, nurses, and other staff and visitors. Many challenging issues arise related to missing information, waiting times, lost appointments, tight schedules and the MSA is the person who must be the liaison between the patient and the clinical staff to resolve situations. In addition, many of the patients and families are not from the local area and this sometimes creates additional anxiety. It is extremely important that the contract personnel maintain a consistent high level of customer service and convey understanding, responsiveness, care, and compassion.
- Patient appointment Activities: Processes patient upon presenting for appointment in accordance with department guidelines. Notifies appropriate care provider of patient arrival. Monitors wait time and intervenes appropriately.
- Reception Activities: Open reception area for workdays; receive direct telephone calls following department guidelines; provide reception; plan for interpreter services and transport services; close reception area. Apply an accurate patient identification wristband to all patients. Respond to medical emergencies in accordance with department guidelines.
- Teamwork: Demonstrates understanding of program mission and completes work to support team goals. Assists other team members in completing assignments, as necessary or as requested, and fills in where and when needed.
- Equipment, Material and Supply Support Activities: Request and stock appropriate medical/pharmaceutical supplies and forms; request and stock office supplies and forms and acquire meals and snacks for patients.
- Scheduling Activities: Schedules and cancels patient appointments in the Clinical Centers and internal department computerized appointment scheduling systems, following procedures set forth in department's guidelines. Prepares daily schedule for providers.
- Admissions Activities: Performs registration and admission activities, updates patient demographic information in the electronic medical record system, arrange patient transport (taxi and ambulance)
- Patient Travel Voucher Activities: Creates patient travel vouchers for air/train travel and travel reimbursement, processes patient travel invoices, and scans documents.
- Clinical Support Activities: Order X-rays as requested, perform monthly safety inspections; maintain patient confidentiality at all times. Assist in preparing patients for consults as appropriate.
- Statistics, Reporting and Other Administrative Activities: Log in consults using department identified tracking programs. Enter data, produce, and/or distribute various reports; document topics discussed in meetings and orientation; collect and enter patient wait and processing time statistics; issue keys and control key inventory as required.
- Knowledge: Contractor should be knowledgeable in the operation of office equipment (computers, copiers, fax machines, labelers, etc.) to scan, file and/or store both electronic and paper documents for future reference. In addition, he/she should be proficient in Microsoft applications (Word, Excel, etc.).

QUALIFICATIONS AND CONSIDERATIONS

These positions are administrative in function but are in a clinical/patient-related setting.

MSA duties include generic clerical skills such as greeting and directing visitors, fielding phone calls, scheduling meetings, responding to callers, taking minutes, composing memos, transcribing, developing presentations, generating reports, and monitoring invoices and expense reports. Must have basic computer skills to conduct internet research and perform data entry. Additional duties may include filing and faxing. A minimum of high school diploma or GED is required.

The Government designee will notify the Contractor of a need for an MSA. The Contractor shall be able to provide staffing for days, and evenings which shall include holidays/weekends and overtime as needed. The Government will provide the Contractor the requested start date, tour of duty days and hours and the location of the assignment. The Contractor shall be able to provide several resumes for review for this need within 5 business days of request. The candidates should have already been pre-screened by the Contractor to be able to meet the requirements. The resumes should include the phone number and email of candidates so that the designated Government employees can conduct a phone, virtual, or in-person screening at the convenience of the Government. The Government shall have the right to select the most appropriate candidate for the position. The Government will conduct a screening of candidates by phone, virtually or in person to determine if the candidates meet the requirements and the Government will make the final selection. The Government will notify the Contractor of candidates that are selected and not selected for a specific position. The contractor should be able to start work, on average, within 2 weeks after the contractor has received notification they have been approved for a NIH ID badge.

The Contractor shall provide replacement coverage for any of their assigned contractors who do not come to work. Any staff considered as replacement staff must have been approved by the Government and have the required NIH Clinical Center training and location specific training to be considered competent to fill this role.

All contract staff assigned shall coordinate their leave requests with the Contractor. It is the responsibility of the Contractor to coordinate staff leave requests. It is also the responsibility of the Contractor to provide an approved and trained replacement when vacancies and/or absences occur. It is the responsibility of the Contract on-site supervisor to ensure daily coverage on all shifts and on holidays and weekends.

The Nursing Department has a mixed staffing model, whereby, both government and contract staff will be providing MSA coverage in the designated locations.

The on-site Contract supervisor will be responsible for coordinating all leave requests with the government supervisors before approving contractor leave. The Contract on-site supervisor must ensure that there is coverage before leave is approved.

The Government supervisors of the area where the contractors are assigned/designee, except in the case of unplanned, emergency leave, must also be notified of approved leave requests and provided with the

name of the replacement contract staff member. In the case of unplanned, emergency leave, the COR/designee and government supervisor of the area must be notified at least 2 hours or as designated by the COR before the start of the contract staff member's tour of duty and must be provided with the name of the replacement contract staff. The government reserves the right to determine the assignment location for replacement staff in order to best meet the needs of the program for that day.

Since the services being provided are in a health care setting, contractors assigned must be able to meet the program needs which may include staying past their assigned work shift until all patients have been seen. The Government has the right to change tour of duty hours for specific areas also based on the requirements of the program in that location.

Service: Full Time/Part Time/Intermittent - Medical Support Assistants

Hours: Days, Evenings, which shall include holidays, weekends and may include overtime as needed

Lunch: Contractor employees shall receive a **thirty (30) minute** unpaid lunch break with no other breaks during their official tour of duty. Breakfast shall be eaten prior to start of employee's tour of duty (**Note:** Lunch times are coordinated by the Contract Officer Representative/designee to ensure appropriate staffing coverage for the area).

Location: National Institutes of Health
Clinical Center, Building 10 Bethesda, MD 20892

The Contractor shall maintain sufficient staffing to meet all performance requirements of this contract and shall ensure that Contractor personnel adhere to Government-approved work schedules.

Contractor personnel shall report for duty at the scheduled start time. A grace period of ten (10) minutes after the scheduled start time is permitted. Arrival beyond the ten (10) minute grace period without prior Government approval constitutes tardiness and a failure to meet contract requirements.

Habitual or repeated use of the grace period, as determined by the Government, may be considered excessive and shall constitute noncompliance with the contract attendance requirements, even if arrival occurs within the ten (10) minute grace period.

The Contractor shall notify the Contracting Officer's Representative (COR), or other designated Government point of contact, of any absence, late arrival, early departure, or other unavailability of Contractor personnel that may affect contract performance. Notification shall be made as soon as practicable, but no later than two (2) hours before the scheduled start time, and shall include the affected individual(s), the anticipated duration, and the Contractor's mitigation plan.

The Contractor shall provide qualified substitute personnel or implement other mitigation measures as necessary to ensure uninterrupted performance of contract requirements. Substitute personnel shall

meet the minimum qualifications specified in this contract and shall be provided at no additional cost to the Government unless otherwise authorized in writing by the Contracting Officer (CO).

Upon identification of attendance or punctuality deficiencies, the Government may require the Contractor to submit a written Corrective Action Plan (CAP) within two (2) to five (5) business days, as specified by the CO or COR. The CAP shall include root cause analysis, corrective actions, and an implementation timeline.

Failure to comply with the requirements of this clause, failure to submit or implement an acceptable CAP, or continued attendance or punctuality deficiencies may be considered a failure to perform in accordance with the contract and may be addressed using the remedies available under the contract as determined by the CO.

CUSTOMER SERVICE

Evidence of excellent customer service is critical to the mission of NIH. The contractor shall provide customer service that:

- Treat everyone as your customer.
- All customers are greeted promptly upon arrival.
- Smile, eye contact, sincere greeting and proactive offering of assistance is the consistent approach with every customer interaction (example: Good morning, how can I help you?).
- Verbal, voice tone, and body language is consistently perceived by customer to be courteous, respectful, and sincere.
- If customer has to wait longer than 15 minutes in the waiting area, an explanation for the wait is provided.
- A positive impression is consistently created for customers as evaluated by reports to supervisor from customers and other stakeholders.
- Appropriately refers issues or concerns to site supervisor in a timely manner.

Actions in the Event of Non-Compliance with the Customer Service Requirements Above: The Government reserves the right to request the removal of any of the Contractor's employees for reasonable cause. The NIH Contract Officer Representative will coordinate as necessary with the NIH Contracting Officer and make such a request only to the Contractor's Project Manager and/or designated representative.

All of the positions covered by this contract involve direct interaction with patients, visitors and staff and it is extremely important that each person present themselves in a professional manner.

HHS/NIH Identification Name Tag Guidelines

An approved identification name tag shall be worn at all times while on duty. Each Contractor employee assigned to this contract will be issued an official HHS/NIH Identification Card upon successful

completion of HSPD-12 background requirements. If unable to obtain clearance, the individual will not be assigned to perform duties under this contract. This official HHS/NIH identification card must be worn at all times while performing services under this contract.

PERSONAL APPEARANCE

Appropriate office attire is required for all contractor staff working at the Clinical Center. Personnel working in all the areas will be interacting with patients, visitors, and staff and it is important that staff dress appropriately for a customer-focused health care setting.

Contract staff shall be prohibited from wearing tee-shirts with inappropriate language (profanity, slang, etc.) and/or offensive images. Cropped tops, spaghetti strap tops, and extremely low-cut tops are also prohibited. Pants with holes or tears and shorts are not acceptable and prohibited. Contract staff may not wear provocative skirts, dresses, or articles of clothing with see-through appliqués at work. Employees clothing must be neat and clean. Tattoos with profanity and/or offensive images shall be covered. Shoes must have closed toes.

Actions in the Event of Non-Compliance with the Guidelines Above: The Government reserves the right to request the removal of any of the Contractor's employees assigned for reasonable cause. The NIH Contract Officer Representative will coordinate as necessary with the NIH Contracting Officer and make such a request only to the Contractor's Project Manager and/or designated representative.

STANDARDS OF CONDUCT

The Government requires a favorable image and considers it to be a major asset to be professional both in conduct and appearance. Each Contractor employee is expected to adhere to standards of conduct that reflect professionalism on themselves, their employer, the NIH and the Federal Government. The Government reserves the right to direct the Contractor to remove an employee from the work site for failure to comply with the standards of conduct or appearance.

The Contractor will be responsible for controlling the employees' conduct and ensure that employees and visitors of the NIH are treated with discretion, respect and courtesy at all times.

The Contractor will be responsible for controlling the employee's timeliness and will ensure that all contractors report to the duty station on time and ready to start work at the start of their tour of duty. The Contractor will ensure that all contractors remain at their duty stations throughout their tour of duty and do not leave their duty stations during their tour of duty without informing the Government's Contract Office Representative or designee.

The Government reserves the right to request the removal of any of the Contractor's employees for reasonable cause. The NIH Contract Officer Representative will coordinate as necessary with the NIH Contracting Officer and make such a request only to the Contractor's Project Manager and/or designated representative.

The NIH Contract Officer Representative will coordinate with the NIH Contracting Officer in order to make all determinations regarding the removal of any employee(s) from the work site. In the event of a dispute,

the employee shall be removed from the contract until there is a resolution of the facts and the NIH Contracting Officer renders a final decision. When the reason for the removal is due solely to misconduct or security on the part of the employee, a replacement contractor shall be provided at the Contractor's expense and not chargeable to the Government.

A determination of "unfit for duty" may be made from, but not limited to, incidents involving the following types of misconduct:

Neglect of official duties, including sleeping while on duty, unreasonable delays or failure to carry out assigned tasks, conducting personal affairs during official time, use of personal cell phones or any personal digital device (for example, Blackberry) while on official duty, personal use of any computer during official duty, participation in any protocol in the hospital while on official duty, falsification or unlawful concealment, removal, mutilation, or destruction of Government property, official records or documents, or concealment of material facts by willful omissions from official records or documents, disorderly conduct, use of abusive or offensive language, flirting or sexually suggestive language or actions, any form of discrimination or sexual harassment, quarreling, intimidation by words, actions or fighting as well as participation in disruptive activities which interfere with the normal and efficient operations of the Government, theft, vandalism, immoral conduct or any other criminal actions, selling, consuming, or being under the influence of intoxicants, drugs, or substances, which produce similar effects, unauthorized use of Government communications equipment or other Government property including, but not limited to telephones, copying/fax machines.

Note that the above listing is not all inclusive.

Criminal Actions: Contractor employees are subject to criminal actions as allowed by law in certain circumstances. These include, but are not limited to, the following:

- Falsification or unlawful concealment, removal, mutilation, or destruction of Government property, official records or documents, or concealment of material facts by willful omission from official records or documents.
- Unauthorized use of Government equipment, property, theft, vandalism or immoral conduct.
- Unethical, improper use or misrepresentation of official authority or credentials.

Contractor employees must conform to all local and other applicable regulations. The Contractor shall take appropriate action in the event an employee(s) become involved with civilian authorities as a result of misconduct or any other investigation, and immediately notify the Government Contract Officer's Representative. Upon notification by civilian authorities of a pending investigation of a Contractor employee(s) the Contractor's Project Manager shall immediately notify the Government Contract Officer's Representative. If the Contract Officer's Representative is not available, notification must be relayed as soon as possible thereafter.

Contractor Email Access: All Contractor staff that have access to and use of NIH electronic mail (e-mail) must identify themselves as contractors on all outgoing e-mail messages, including those that are sent in reply or are forwarded to another user. To best comply with this requirement, the contractor staff shall set

up an e-mail signature ("AutoSignature") or an electronic business card ("V-card") on each contractor employee's computer system and/or Personal Digital Assistant (PDA) that will automatically display "Contractor" in the signature area of all e-mails sent.

Personal Electronic Device Use While on Duty: Contractor personnel shall devote their full working time and attention to performance of the contract. Use of personal electronic devices (including mobile phones, smart phones, tablets, or similar devices) for non-contract-related purposes—such as texting, personal calls, social media, streaming media, or viewing television or videos—while on duty is strictly prohibited.

Limited use of personal devices may be permitted by the Contractor for bona fide emergencies or as otherwise approved by the Contractor, provided such use does not interfere with contract performance, workplace safety, or Government operations. The Contractor is solely responsible for establishing, communicating, and enforcing policies governing employee conduct and ensuring that time billed to the Government reflects only hours worked in direct support of contract requirements.

OPERATION DURING INCLEMENT WEATHER, GOVERNMENT CLOSURE, AND EMERGENCIES (MAN-MADE OR OTHERWISE)

All Medical Support Assistants are considered “emergency essential” personnel under the NIH Clinical Center policy, "Designation of Emergency and Non-Emergency Employees for Periods of Inclement Weather or Other Temporary Closings." Contractors will be required to be available, ready, willing and able to report for duty under all circumstances (e.g., inclement weather, government closings and holidays, lapses in appropriations, non- emergency and emergency situations) The Clinical Center is operational 24 hours a day/7 day a week/365 days a year.

It is the Contractor’s responsibility to ensure their staff report to work to support any non- cancelled operations during inclement weather, even if the Government has announced Official Closure or Liberal Leave. The Contractor(s) will be required to staff their positions.

The Contract Officer Representative (COR)/Designee shall notify the Contractor if certain areas of the Clinical Center are closed, and contractor staff are not required to report to work or can leave early.

MANDATORY TRAINING ORIENTATION

Each person selected to work at the Clinical Center must receive an orientation to the position and the Clinical Center. The government will provide the training as designated by the CC Office of Workforce Management and PSSD. Attendance by all contractor employees is mandatory. In addition, the government may add additional training requirements during the contract period. CC training shall be completed within 30 days of the initial start date. Documentation for completion shall be sent from the contract staff to the Government COR during initial orientation and annually as required.

PATIENT CONFIDENTIALITY AND PRIVACY ACT COMPLIANCE

The patient has the right to privacy concerning their medical care. The patient has the right to expect that all communications and records pertaining to their care are treated as confidential. The Contractor’s

employees shall agree to comply with the provisions of the Privacy Act of 1974, as amended, and the agency rules, regulations and policies implementing the Act.

Any patient data identified/utilized/transmitted will be subject to the Privacy Act and confidentiality requirements under which the Clinical Center operates. All information pertaining to specific Clinical Center patients is protected from unauthorized use or disclosure by the Privacy Act of 1974. Violation of the Act may involve the imposition of criminal penalties.

INFECTIOUS DISEASE COMPLIANCE REPORT

All employees covered under this contract shall follow all the rules and regulations for infectious disease control.

Medical support assistants must ensure that they comply with all applicable federal and state government regulations regarding control of infectious diseases in health care facilities (Note: This requirement does not apply to offsite non patient employees). Contractors should consult the specific regulations for further details, exemptions, documentation, and record keeping requirements. Specific examples include, but are not limited to:

Contractors must ensure that they comply with all applicable federal and state government regulations regarding control of infectious diseases in health care facilities. Contractors should consult the specific regulations for further details, exemptions, documentation, and record keeping requirements. Specific examples include, but are not limited to:

- **Rubeola** - Hospital workers must have proof of rubeola (measles) immunity. Workers born after 1956, working at least 20 hours each week, who are newly retained as medical staff, direct or contractual employees, or volunteers shall have documentation of receipt of either one dose of live measles virus vaccination or proof of immunity by blood test for antibody to rubeola. The contractor shall keep the measles vaccination or immunity status of each worker on file.
- **Hepatitis B Vaccine** - Workers with occupational exposure to blood or other potentially infectious materials shall be offered the hepatitis B vaccine unless the worker has previously received the complete hepatitis B vaccination series, antibody testing has revealed that the worker is immune, or the vaccine is contraindicated for medical reasons. Workers who decline to accept the hepatitis B vaccination must sign the mandatory declination, as specified in the OSHA standard. The contractor shall maintain worker records for at least the duration of employment plus 30 years, in accordance with the standard. (29 CFR Part 1910.1030, OSHA Occupational Exposure to Bloodborne Pathogens).
- **Universal Precautions Training** - Workers with occupational exposure to blood or other potentially infectious materials must receive annual training in Universal Precautions. For workers in the Clinical Center and/or credentialed to provide patient care, this training is routinely offered by the Hospital Epidemiology Service (HES). Contractors must either provide annual training to their employees with occupational exposure or ensure that they receive training through the HES.

Contractors must maintain training records for three years, according to the standard. (29 CFR Part 1910.1030, OSHA Occupational Exposure to Bloodborne Pathogens).

- **Occupational Exposures to Blood or other Potentially Infectious Materials -** Workers with occupational exposure to blood or other potentially infectious materials shall be offered post-exposure evaluation and follow-up. The contractor shall maintain these records for at least the duration of employment plus 30 years, in accordance with the standard. (29 CFR Part 1910.1030, OSHA Occupational Exposure to Bloodborne Pathogens). The NIH Occupational Medical Service (OMS) provides emergency clinical evaluation and treatment to NIH employees, non-NIH workers, and visitors on campus who report an exposure to potentially infectious materials. If the exposure involves a primate retrovirus, OMS also offers chemoprophylaxis and serologic monitoring. Contractors are responsible for follow-up of exposures other than those involving primate retroviruses.
- **Mycobacterium Tuberculosis -** Workers who have the potential for occupational exposure to *M. tuberculosis* shall receive counseling, screening, and evaluation. These workers include: persons who routinely have patient contact or who enter patient rooms, examination or treatment rooms whether occupied or not; persons who are exposed to *M. tuberculosis* in a laboratory or morgue; or persons who work in a primate animal care setting. Workers who have positive tuberculin skin test (TST) results or a positive interferon gamma release assay (IGRA), TST or IGRA conversions, or symptoms suggestive of TB should be identified, evaluated to rule out a diagnosis of active TB, and started on therapy or preventive therapy if indicated. The results of pre-placement and periodic TST or IGRA testing (if indicated) must be kept in a retrievable aggregate database by the Clinical Center and on file by the Contractor. Contractor data must be reported to the HES as requested. (Centers for Disease Control and Prevention. Guidelines for preventing the transmission of *Mycobacterium tuberculosis* in health-care facilities, 2005. MMWR 54 RR 17).

Any contract employee physically working in Building 10, irrespective of whether or not the individual may have occupational exposure to M. tuberculosis, must have a pre-placement TST skin test or IGRA and those with positive TST or IGRA must be evaluated as described above.

- **Varicella -** Contract workers with patient contact must have a pre-placement evaluation for history of chickenpox (varicella) and/or receipt of varicella vaccine. The contractor shall keep the varicella history and/or vaccination status of each worker on file.
- **Influenza –** Any contractor or contractor employee physically working in building 10 from October through April, shall be required to either be immunized annually with the influenza vaccine or sign a declination form (e for the current year. Records of vaccination or declination shall be kept by the contractor for a period of one year and made available to the HES upon request. (Prevention and Control of Influenza

REQUIRED CONTRACTOR DOCUMENTATION (NOTE: The Government does not pay for security clearances, physicals, or related requirements that are required for personnel to be accepted for agreed upon assignments):

The Government will provide the Contractor with a check list for each new contractor accepted for an assignment. The Contractor must provide the written information (check-list and supporting documentation) at least 5 business days prior to the assignment. The documentation will be reviewed by Government designee within 48 hours of receipt. The Government designee will confirm that all required documentation has been received prior to the start date. In the event that the documentation is not complete or there is an issue with any of the documentation, the Contractor will be notified and the start date may be delayed or terminated.

The Agency Checklist will be submitted to the Contracting Officer Representative (COR). The checklist will be provided upon award.

NOTE: The Contract Agency will need to provide the legal spelling of the contractor's name, phone number and email, in order to begin the process for the issuance of a government contractor ID. Social Security numbers cannot be provided by e-mail.

The Contract Agency must provide a copy of the following information, in addition, to noting compliance on the check-list form.

- Current resume and 2 professional references (1 must be from a supervisor)
- Criminal background check for Maryland, Virginia, and the District of Columbia. (must be current within 30 days)
- Non-Disclosure Statement
- Medical Requirements
 - Physical Exam (completed within the year, cleared for full duty with no physical restrictions)
 - PPD (within past 6 months)
 - Hepatitis B Titer (must show evidence of vaccine/titer levels or signed declination form)
 - Chicken Pox (must show evidence of vaccine/titer levels indicating immunity status)
 - MMR (must show evidence of disease or vaccine/titer levels indicating immunity status)
 - Influenza Vaccine (must show evidence of vaccine given or medical documentation from physician as to why the vaccine can't be given)
 - Drug Screen (within 30 days prior to assignment)

QUALITY CONTROL AND MONITORING

Quality Control will be monitored by the Government by conducting periodic review of performance and reviewing customer complaints.

The Contractor and the NIH Clinical Center will review performance of the Medical Support Assistant staffing services for meeting the terms and conditions of this contract. This will be accomplished through weekly/bi-weekly or monthly status/progress meetings between the NIH Clinical Center personnel responsible for managing and administering this contract (Contracting Officer and Contract Officer's Representatives and the on-site contract supervisor. The frequency of the meetings will be determined by the CORs.

At these meetings, the Government will convey problems/difficulties reported by contract users and recommended solutions will be discussed and mutually agreed to between the Government and the Contractor. The Contracting Officer will be kept fully informed of any contract performance problems so that appropriate action may be taken. The Contractor will have the ability to bring up any issues and the Government

will review the concerns and provide a response as long as it doesn't require a modification to the contract requirements. Any recommended changes to the contract will be discussed with the Contracting Officer.

Quality Control Program

The Contractor shall develop a proactive Quality Control Program for measuring and attaining quality of performance under this award. The Contractor's Quality Control Program shall explain the manner in which the Contractor shall ensure all requirements are being accomplished. A sustaining focus throughout the Quality Control Program shall be the attainment of continuous quality improvement. The Contractor shall demonstrate a concerted effort in improving its QCP to prevent unsatisfactory performance from consistently recurring in any area and to ensure unsatisfactory performance is addressed and rectified in a timely manner.

The Contractor will be responsible for responding, in writing, to any events/complaints involving a contract employee within 24 hours of notification of the event/complaint or next business day. If the issue requires immediate response and resolution, the Contractor will verbally inform the COR and follow-up with a written response.

Quality Assurance

The NIH Government Contract Officer Representative(s) will inspect for compliance with contract terms throughout the performance period. Typical procedures include random sampling, planned sampling, scheduled inspections, incident inspections, and validated customer complaints. The Contractor shall provide documentation of adherence to the Plan upon request. The Government will sample verify differential application at least quarterly; recurring non-compliance shall be subject to corrective action plans and possible invoice adjustments.